10-11

sounds. Tics come and go over time, varying in type, frequency, location, and severity. The first symptoms Tourette's patients experience usually occur in the head and neck area, though they may progress to include muscles of the trunk and extremities. The symptoms

10-12

of Tourette's are involuntary, but there are some people who can sometimes suppress, camouflage, or otherwise manage their tics in an effort to minimize their impact on functioning. However, people affected by Tourette's often report a substantial buildup in tension when suppressing their tics to the point where they often

10-13

feel that the tic must be expressed. Tics in response to an environmental trigger can appear to be voluntary or purposeful but are not. Many people with Tourette's experience additional neurobehavioral problems that include inattention, hyperactivity and

10-14

impulsivity. People with Tourette's have also reported problems with depression or anxiety disorders, as well as other difficulties Tourette's syndrome may or may not directly cause. Tourette's is a diagnosis doctors make after verifying that the patient has had both motor and vocal

10-15

tics for at least one year. The existence of other neurological or psychiatric conditions can also help doctors establish the diagnosis of Tourette's is the correct one. Knowledgeable clinicians consulted by patients do not often misdiagnose common tics. However, atypical

10-16

symptoms or atypical presentation such as onset of symptoms in adulthood may require specialty expertise for diagnosis. There are no blood or laboratory tests needed for diagnosis. However, neuroimaging studies, which include magnetic resonance imaging, computerized

10-17

tomography, and electroencephalograms, as well as certain lab tests may be used to rule out other conditions that Tourette's might be confused with. In terms of the role of genes in the disorder, evidence from twin and family studies suggests that Tourette's is an inherited disorder. Although

10-18

early family studies suggested an autosomal dominant mode of inheritance characterized the condition, more recent studies discovered the pattern of inheritance was much more complex. Although there may be a few genes that have substantial effects, it's also possible that many genes with smaller effects and environmental factors may

10-19

play a role in the development of Tourette's. Genetic studies also suggest that some forms of Attention Deficit Hyperactivity Disorder and Obsessive Compulsive Disorder are genetically related to Tourette's, but there is less evidence for a genetic relationship between Tourette's

10-20

and other neurobehavioral problems that Tourette's commonly co-occurs with. An important role in Tourette's gene expression is played by the sex of the person. In particular, males affected by the gene are more likely to have tics and females who have a genetic predisposition are more likely

10-21

to have obsessive-compulsive symptoms. Much current research is aimed at better understanding the genetic basis and symptomatology of Tourette's, as well as its relationship to related disorders. The hope is that this research leads to better diagnostic tools and better

10-22

treatments for Tourette's.

Back
13

Back
14

Back
15

Back
16

Back
17

Back
18

Back
19

Back
20

Back
21

Back
22

Back
23

Back
24

Back
25

Back
26

Back
27

Back
28

Back
29

Back
30